Nivolumab every 2 weeks for head and neck cancer

Nivolumab aims to control head and neck cancer that has returned or spread, relieve symptoms and help some people live longer. Benefit and side effects are reviewed for you.

Recurrent or metastatic squamous cell carcinoma of the head and neck after platinum treatment; the cancer cannot be treated with curative local therapy and has progressed or returned within six months of the last platinum dose. · NCCP 00483 v13a · Document revised 9 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Recurrent or metastatic squamous cell carcinoma of the head and neck after platinum treatment; the cancer cannot be treated with curative local therapy and has progressed or returned within six months of the last platinum dose.

My overall treatment

Nivolumab alone every 2 weeks; duration individually reviewed.

14 day view

Treatment timing in this window

Nivolumab treatment — Day 1

Nivolumab monotherapy

Nivolumab is given as 240 mg through a vein over 30 minutes OR 600 mg as an injection under the skin over 3–5 minutes, on Day 1 every 2 weeks. Your team selects one formulation; these doses are not interchangeable milligram for milligram. Allow extra time for safety checks, preparation and observation. Treatment continues until the cancer progresses or side effects become unacceptable, with individual benefit and safety review. There is no fixed number of cycles in this head and neck treatment course.

A typical 14-day cycle

Day 1	Day 2–14
Treatment visit	Between visits

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline safety and preparation** NCCP	Full blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune disease, transplants, previous immune side effects and all medicines.
During	**Bloods, symptoms and support** NCCP	Full blood count, kidney and liver tests and glucose before every 2-week cycle. Thyroid tests once a month and when indicated; cortisol when clinically indicated.
After / response	**Follow-up and tests** NCCP / individual plan	Keep reporting new symptoms after treatment ends. Immune side effects require monitoring for at least five months after the last dose. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.

Between treatments

- Immune side effects may start between visits or after the final dose. Report changes promptly.
- A longer gap between treatments does not remove scheduled tests or the need to report symptoms.

Bring your medicine list, test dates, treatment calendar and actual urgent contacts.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / nervous system

Nivolumab

Severe headache, confusion, weakness, dizziness or marked thirst need prompt advice.

3 Heart

Nivolumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

5 Kidneys

Nivolumab

Report much less urine, new swelling or a change in kidney tests.

7 Skin

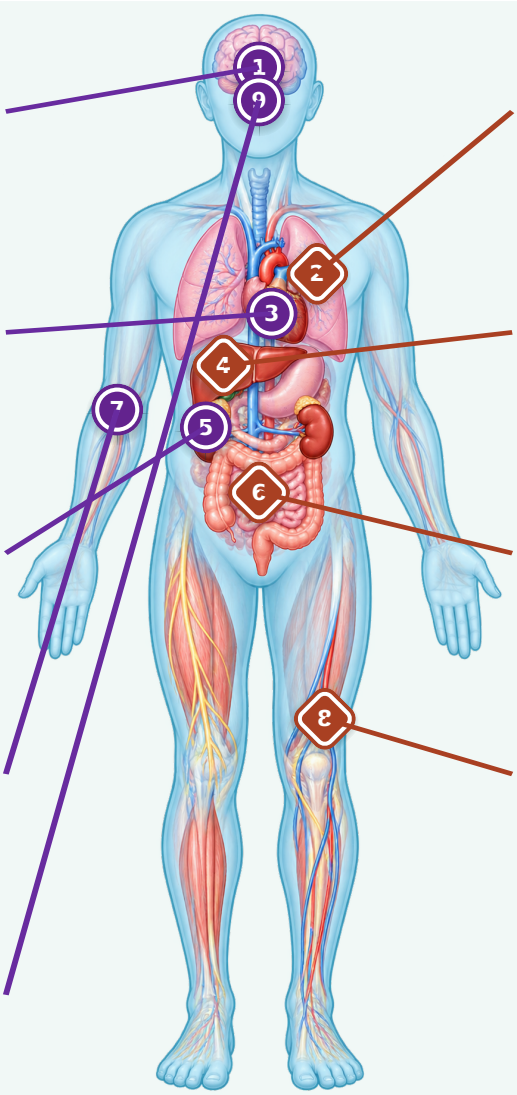
Nivolumab

Report a spreading rash, painful skin, blisters or peeling promptly.

9 Eyes / vision

Nivolumab

New blurred vision, painful eyes or visual change needs prompt advice.



2 Lungs

Nivolumab

New cough or breathlessness can signal inflammation or another urgent cause.

4 Liver

Nivolumab

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

6 Bowel

Nivolumab

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

8 Muscles / nerves

Nivolumab

Severe weakness, drooping eyelids, swallowing or breathing difficulty can be urgent immune effects.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for new breathlessness, chest pain, diarrhoea, severe tummy pain, yellow skin or eyes, much less urine, severe headache, vision changes, marked weakness, confusion, unusual thirst or a spreading or blistering rash. Severe breathing difficulty, collapse or seizure is an emergency.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Nivolumab · head and neck

Every 2 weeks; individual duration

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record only; these boxes do not define a fixed treatment course.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.